

‘But we want cassava, not COVID-19 vaccines’: the politics of vaccination in Uganda

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Early in 2020, Uganda’s local and national media began to cover news of COVID-19 deaths. While these deaths were occurring in distant places such as in China, Italy and the United States, television and print media showed that COVID-19 was dangerous and virulent. News broadcasts were frequently coupled with messages that Ugandans and people in other African nations must respond appropriately to the anticipated spread of the disease (Sirleaf [2021](#); Africa CDC [2020](#), [2021](#)).

The World Health Organization (WHO) declared COVID-19 a global pandemic on 11 March 2020. On 14 June 2020, the cases detected and reported on the African continent represented only 3 per cent of total cases worldwide (Morrison 2020). Uganda’s first case of COVID-19 was confirmed on 18 March 2020 – a traveler arriving at Entebbe International Airport. Thereafter, Points of Entry (PoEs) became hotspots for cases. These were mostly asymptomatic and mild (Kanyike et al. [2021](#); Bongomin et al. [2021b](#)), but, nevertheless, the state required all PCR¹-diagnosed positive cases of COVID-19 to quarantine for 14 days.

The president, Yoweri Museveni, led disease containment measures, assisted by a team of ministers and other politicians appointed to oversee activities that mirrored a globally standardized repertoire of containment measures: enhanced surveillance, laboratory testing and subtyping, risk communication and community engagement, logistics and supply chain management, infection prevention and control and case management. Lockdowns were the preferred method implemented to limit transmission (see Akello [2020](#)), and the country experienced two stringent lockdowns which, combined, extended from March to July 2020. People were under regular surveillance and punished when they breached lockdown measures; when they tested positive – even when

asymptomatic – they were quarantined or hospitalized, often forcibly, for 14 days. Schools, places of worship, bars and other places of entertainment were closed and public transport was restricted in a bid to halt the spread of the virus (Parker et al. 2020; Burt et al. 2021). By October 2021, Uganda had been through one of the longest lockdown periods globally.

On this basis, strict lockdown was already seen to have worked, and a major spread of infection had been averted. According to the Government of Uganda, by 30 September 2020, Uganda had seen a total of 8,129 COVID-19 cases with 4,260 recoveries and 75 deaths – hence a Case Fatality Rate (CFR) of less than 1 per cent of a population of slightly under 46 million (Margini et al. 2020). Kampala metropolitan area was most affected, with 2,735 total cases and 47 deaths. There were localized upcountry district outbreaks, with 104 of 107 districts reporting 87 cases between them and no deaths in September 2020. Nine months later, the vaccination program commenced.

As I illustrate below, the Ugandan state exerted excessive power and authority to enforce disease containment measures. When COVID-19 vaccines were introduced, in June 2021, many people hesitated to access them. The political environment and the authoritarian nature of the enforcement of pre-vaccination containment measures influenced people's distrust of vaccines. There was also, at that time, a need to promote vaccine equity from African actors such as Africa CDC. Donor agencies were pressing for vaccination targets. Actors' pressures overrode engagement with local epidemiologists and little attention was paid to emerging experiential evidence. This led to a disjuncture between government campaigns to increase vaccine uptake and local attitudes, including among people being urged to present for the vaccine, who viewed the disease as mild and failed to see the relevance of vaccination.

Field sites and methods

In this chapter, I analyze the government's endeavor to ensure population acceptance of COVID-19 vaccines. I describe state heavy-handedness in forcibly hospitalizing asymptomatic cases and enforcing prolonged lockdown and containment measures, all of which contributed to local people's resistance to control measures and the vaccine program. I analyse too how global health partners' focus on metrics had unforeseen negative consequences on vaccine uptake. I examine these issues following events in our study sites.

I draw on multi-sited ethnographic research conducted from June 2020 to September 2022 in the districts of Kasese, western Uganda; Gulu, northern Uganda; and Kampala and Wakiso districts in central Uganda. Below, I document the perspectives of frontline health workers working in urban and rural areas, and district officials in selected areas. To document the perspectives of national actors who led COVID-19 uptake campaigns, I participated in 17 national level meetings for COVID-19 containment, and I conducted over 450 interviews with officials working for the Ministry of Health (MOH), clinicians, members in the District, Hospital and Sub-County Task Force for COVID-19, National Emergency Operation Centre (NEOC) and National Task Force (NTF) members in Kampala. With consent, adults were interviewed about their experiences with COVID-19, how they interpreted containment measures, and whether they accepted vaccination. Handwritten and sometimes recorded interviews were later typed for coding, further classification and identification of recurring themes.

Containing asymptomatic COVID-19

National policy

Between March and September 2020, in his frequent nationwide addresses on epidemic preparedness and response measures, President Museveni stressed the need to isolate and hospitalize asymptomatic cases. Positive cases needed to be isolated to prevent the spread of disease; **if people who tested positive did not adhere to these instructions, the army and the police would be called upon to ‘discipline’ them, with the threat of incarceration.** Surveillance teams of the military, police and local authorities were deployed nationwide, particularly at PoEs (Parker et al. 2022). These teams were armed. They enforced lockdown and handwashing, and had ambulances to transport confirmed cases to designated points for disease management and hospital admission. The tone in presidential speeches was militaristic and authoritarian. COVID-19 was regarded as

... an invisible enemy, and people need to lie down, take cover, and listen, discover what the enemy is doing, how it is behaving. And this was the purpose of lockdown. But if the health teams discover that you have been attacked, and especially truck drivers need to cooperate, allow to be isolated and treated at designated health centers. Any resistance will be regarded as a case of indiscipline. (Museveni, 17 March 2020)

Surveillance teams of the military, police and local authorities were deployed nationwide, particularly at PoEs. These teams were armed. They enforced lockdown and handwashing, and commissioned ambulances to transport confirmed cases to designated points for disease management and hospital admission.

When patients were discharged from hospital, senior figures of public authority (including Resident District Commissioners) gave speeches exhorting them to 'keep their status negative' by adhering to preventive activities: maintaining a social distance of two meters, constantly sanitizing and wearing face masks. In this context, one respondent reflected on the contradictions of everyday containment activities, particularly when people were forced to stay at home:

It was during lockdown that we saw the highest number of clients – depositing large sums of money. The idea that COVID-19 existed and was killing many people was distant. We never heard or even saw a really sick person suffering from COVID-19 for all the eight months. Trust me, we were essential workers moving around everyday and watching how health teams conducted their business. (Interview, October 2020, Gulu).

Gulu

The designated COVID-19 treatment centre in Northern Uganda was in Gulu. All travelers, villagers and essential workers with positive PCR results were forcibly hospitalized at Gulu Regional Referral Hospital (GRRH hereon), even when asymptomatic or only mildly affected (Akello 2020). Typically, COVID-19 surveillance teams set up offices at PoEs and designated hospitals. Through routinized surveillance and testing, nasal swabs were taken from truck drivers, travelers and essential workers. Because PCR testing took 12–48 hours depending on 'client load' at laboratories, some truck drivers' results would be available only after they had traveled for more than 48 hours. Phone calls would then be made to surveillance teams stationed in districts where the traveler could have arrived, with instructions to immediately intercept and transport them to the nearest COVID treatment center. Three truck drivers who were arrested and detained had traveled a distance of over 500 km, and were subsequently admitted to GRRH for 14 days:

We were arrested at a trading center near Adjuman town. The COVID [-response] team found us about to set off for Juba [South

Sudan]. They surrounded us. They ordered us to leave everything and co-operate with them. They said we had corona and that we were being taken for treatment. When we told them we were not feeling any sickness and we wanted to go on with our business, they threatened us (with arrest at gunpoint). We could not argue more because the ambulance was already parked there. That is how we were ordered to board, brought here and have been here [GRRH] doing nothing. Sometimes the health workers would tell us to sing, to dance and to drink a lot of water (Group interview, July 2020).

In interviews conducted in June 2020, people consistently spoke of the inconvenience of their admission, and they experienced state representatives and disease containment measures as heavy-handed, oppressive and confusing. Surveillance teams forcibly arrested any confirmed case and transported people via ambulance to the nearest treatment center, and the military and police were deployed at treatment centers to prevent violence, resistance and escape of these patients. By July 2020, *people in villages had started using the enforcement teams to arrest and detain 'social deviants' such as land grabbers, alleging that they were travelers and had COVID-19 symptoms* (Akello 2020). Thus, with the securitization of COVID-19 containment efforts, there was a blurring between the experience of the disease and other complex social, economic and political matters.

Compulsory hospitalization had initially been advised by the WHO, but this changed rapidly to focus on the home management of mild cases. Observation and interviews from May to September 2020 with hospital task forces in different designated treatment centers confirmed all cases were asymptomatic. By August 2020, staff attending hospital management meetings at GRRH and other rural COVID-19 management centers were questioning mandatory hospitalization, and were advocating for a return to 'traditional' clinical practice, whereby only critically ill patients were to be hospitalized. They argued that hospitalizing asymptomatic cases was costly and required many clinicians, at a time when the hospital was understaffed. The minister had promised UGX80,000 (around GBP17.6) per day for each member of hospital taskforce teams, but payments were delayed and by October had ceased ('UMA raises concern' 2020; Ajuna 2021).

Asymptomatic COVID-19 patients also resisted containment measures and disease response activities, and found creative ways to escape from hospitals and quarantine centers. Typically, they requested permission to leave to buy water or food in the hospital neighborhood,

and never returned. Surveillance was subsequently heightened. Many patients resisted admission because they could not meet the proposed admission and quarantine costs. In September 2020, in acknowledging this and because hospitals were underfunded, clinicians increasingly declined to engage in non-traditional clinical care by admitting asymptomatic patients in regional and national referral hospitals. In response, the National Task Force proposed referring all asymptomatic patients to the National Centre at Namboole Stadium, established as a field hospital for people with COVID infections, with people charged for their stay.

One event documented in September 2020 involved 20 asymptomatic patients from Kitgum district in northern Uganda. Kitgum district hospital was not a government-approved centre for COVID-19 management and its clinicians avoided hospitalizing asymptomatic patients. Patients were therefore referred to neighboring GRRH for management. The GRRH Hospital Task Force had been directing its limited resources and 70 per cent of staff to care for positive COVID-19 cases, including those forcibly admitted. When another 20 asymptomatic cases arrived at GRRH, members of the task force and other clinicians referred them to the Namboole Stadium, where they were supposed to self-fund 14 days of quarantining and conservative management. Patients burnt down the bus 7 kms out of Gulu city. The police did not identify the cause of the fire and did not identify the patients who fled for safety.

Kasese

Uganda's PoEs, including Kasese district which borders the Democratic Republic of Congo in south Western Uganda, were portrayed as hotspots for COVID-19. In all hotspots, schools with hostel facilities, running water and electricity were approved by the government and guarded by the military to ensure that confirmed cases of COVID-19 and people with flu and cough were quarantined. Only senior officers, including district health officers, designated physicians and the Resident District Commissioner (the political representative) had access to these centers; researchers, journalists and relatives of forcibly quarantined people were prohibited. In September 2020, following interviews with administrators of schools where suspected cases were quarantined, it became apparent these were managed like detention centers, with containment an excuse to arrest and criminalize those who were perceived to be acting against the principle of lockdown in disease containment. One school administrator in Kasese put it thus:

The way I saw it, the people whom the COVID-19 response team brought here for quarantine were not sick at all. They were arrested at small border points or *panyas* [back roads] to DRC [Democratic Republic of Congo], carrying their goods, and trying to find markets in DRC. Some were known FDC [opposition party] supporters, and they were arrested arbitrarily. Some were having a walk, or [were] in their homes.

Cross-border traders who resisted state guidelines to ‘stay at home’ and isolate, and instead crossed into DRC to generate income, were arrested and detained at these quarantine centers, where they divided up cooking and cleaning duties, and spent entire days playing cards together and discussing how to escape. They often ‘escaped’ to conduct small-scale trade, and then returned to their quarantine centers until they were able to receive certificates of discharge. During an interview, a clinician in Kasese mentioned that ‘it was easier to discharge law-abiding citizens compared to (people who were) stubborn, resistant and people questioning authority’. Clinicians and political authorities deliberately detained defiant clients for a longer period of time.

As in Gulu, many villagers in Kasese perceived COVID-19 as a ‘disease of the radio’. For instance, one Village Health Team (VHT) community worker in Karambi sub-district observed:

In this village, we have not seen anyone suffering from COVID. We have seen many dying in far countries like China. In radio announcements, they keep talking about deaths due to COVID, in China and America. This is a disease of the radio and TV (February 2023).

And in a group discussion of seven villagers in Kisanga in February 2023, they asked:

Why has the government continued to announce lockdown, when we have not seen anyone with the disease here? For us this disease is only on radio. We hear it is killing people far away like China, but it has not reached here.

Introducing vaccines in Uganda

In March 2021, Uganda received its first batch of vaccines – 864,000 doses of Oxford-AstraZeneca – through the COVID-19 Vaccines Global

Access (COVAX), a global initiative aimed at the equitable access of COVID-19 vaccines. The Ugandan government announced priority groups to receive the vaccine: health care workers, security personnel, teachers, humanitarian and frontline health care workers and patients living with diabetes, heart disease and HIV/AIDS (Uganda 2020). By October 2021 Uganda had received 5.7 million doses, either through donations or by direct purchase. But from the outset, it was clear that most cases reported in Uganda were mild, and Ugandans were skeptical about the vaccine. People with higher levels of education, with a substantially higher level of knowledge about the vaccine, were more likely to accept it. Even so, many people, including 30.7 per cent of medical students, hesitated to access the vaccine (Bongomin et al. 2021b; Olum et al. 2020; Kanyike et al. 2021), influenced by negative information about the vaccine (53.5 per cent); fears that it might be unsafe (64.4 per cent); and concerns that it was not sufficiently efficacious to prevent disease (36.2 per cent) (Kanyike et al. 2021). Kanyike and colleagues (2021) also showed that around a third of medical students perceived only a slight risk of getting COVID-19, and were not worried about the disease. Other studies also linked low perceived risks associated with COVID-19 to vaccine hesitancy (Bongomin et al. 2021a; Sallam et al. 2021; Jain et al. 2021).

While national records indicate a 70 per cent uptake for vaccines, on 21 April 2021, Abet (2021) reported that more than 280,000 doses of the vaccine were likely to expire before being used, due to vaccine hesitancy and low uptake rates, particularly in upcountry districts where COVID infections were perceived as neither severe nor lethal. The demand for vaccines increased with the onset of the Delta variant in July 2021, mostly among elite groups in major cities like Kampala and Mbarara, by which time any vaccines available in the country had expired.

Again on September 23 ('67,000 Covid-19 vaccine doses' 2021), it was reported that 67,000 vaccine doses from a batch of 128,560 doses donated by Norway were likely to expire, 12,000 of which were in Kampala and 8,000 in Arua district in West Nile. At the time of their receipt by the Uganda Drug Authority, the vaccines had only 10 days left before expiry, and the receipt of drugs being delivered near expiry date diminished people's confidence in their efficacy and reduced their motivation to be vaccinated. This prompted the President to order District Resident Commissioners, District Health Officers and Chief Administrative Officers to carry out mass mobilization for vaccination for priority groups, on threat of dismissal if they failed to do so (President's Speech, Wednesday 22 September 2020) (Museveni 2020).

The President's threats to dismiss Resident District Commissioners and district authorities if vaccine acceptance remained low served to increase vaccine hesitancy and sparked fear among district authorities. In Kasese district, as a result, district leaders, backed by the police, instituted road blocks and forcibly vaccinated travelers and small-scale traders.

On 18 October 2021, Uganda's Minister of Health confirmed that most of the country's vaccines had expired (Kyeyune 2021). Although Uganda had received 5.7 million doses of vaccines through donations and direct procurement, only 2.4 million had been used, with the rest lying idle in storage. Of those doses administered, only 584,066 people out of the country's population of 46 million were fully vaccinated; 1.8 million had received a single dose of either AstraZeneca, Sinopharm or Johnson & Johnson vaccines. This was only 5 per cent of the population targeted to receive the vaccine by the end of September 2021 (Kanyike 2021); fewer than 15 per cent of targeted teachers, 34 per cent of targeted health workers and 19 per cent of targeted security personnel had been fully vaccinated. Kanyike (2021) concluded that Uganda had failed to reach the September 2021 target set by the WHO for countries to vaccinate at least 10 per cent of population priority groups.

In January 2022, local media reports showed that Uganda was set to destroy 400,000 doses of expired COVID-19 vaccines, when less than half of the targeted 22 million people had been vaccinated (Ojambo 2022). In June 2022, evaluating the campaign mode, the Minister of Health confirmed that the expired AstraZeneca and Moderna vaccines had been transported to northern Uganda but had not been used. Of the 500,000 doses of Moderna sent to northern Uganda, only 100,000 doses were used. In mid August 2022, it was projected that 1.2 million doses would have to be destroyed if not used by the end of the month (IntraHealth International 2022).

Rollout

Forcibly vaccinating travelers and small-scale traders ignited a general public outcry and threats of litigation of the state by affected citizens on radio talk shows. This prompted the state and global health partners such as the MOH-Uganda National Expanded Program on Immunisation (UNEPI) office in Uganda to draft guidelines to support a health promotion campaign to enhance vaccine uptake. In campaign mode, with legal backing, COVID-19 vaccines were extracted from routine vaccine supply chains, and people were asked to access vaccines outside health centers.

As described above, the Ministry of Health (MOH) in Uganda received its first donation of 864,000 doses of AstraZeneca COVID-19 vaccine, shipped via the COVAX facility, on 6 March 2021 (WHO 2021). To administer one vial of AstraZeneca, a minimum of 9 to 12 people had to visit a designated vaccine center, since an open vial could not be stored for future use. In June 2021, Uganda received a second batch of vaccines donated by the French government (Agaba 2021); other vaccine deliveries were donated by and acquired through bilateral purchases from China and India's Serum Institute. From receipt of the first batch of vaccines at the Ministry of Health headquarters in Kampala and in other cities like Gulu, Mbarara and Arua, the government's message was received with skepticism. The Ministry needed to use these supplies urgently, and it did this by issuing an order that all men in uniform, particularly in the Uganda People's Defence Forces, were to be vaccinated. Along with military personnel, those people whom one officer at the Ministry of Health called 'vaccine enthusiasts' (people who willingly accepted any vaccines as life-saving technologies) were vaccinated. But District Task Force staff in Gulu and Kasese, and National Task Force officials in Kampala, indicated that much of the first batch of COVID-19 vaccine had expired and had been reported to be withdrawn from health facilities and destroyed by the National Medical Stores; by August 2021, more than 5.6 million doses purchased through a World Bank had expired and were to be destroyed (Bamulanzeki 2021).

COVID-19 containment activities like lockdown shaped responses to vaccination, and as noted above, the roll out of the program was largely met with resistance, low uptake and suspicion. One officer told Ugandans:

If you have not yet been vaccinated, please make sure you do so while we still have some doses. In the coming months, no one will be allowed to cross PoEs, enter public offices and banks, unless you have proof for COVID-19 vaccination. (Interview, June 2021)

During a group interview (May 2022), villagers in Gulu said, and others agreed: 'For sure, let the government invest this money they are using on Covid to buy and distribute cassava instead'. An officer at the NEOC similarly stated: 'Give me cassava any day. It is because I am sure of its purpose, in feeding the hungry. But this Covid response thing has divided many people even here in NEOC' (Interview, May 2022).

Travelers at major airports globally needed to have proof of vaccination against COVID-19. In Uganda this was enforced at various

PoEs, including Kasese, where travelers received extra doses to the required two doses if they had left their vaccination cards at home or had not received a vaccine card when vaccinated. One 56-year-old man traveling from Kasese district reported:

In one week, I was vaccinated two times ... because I was traveling to Kampala yet I had no vaccine card. Instead of missing the business opportunity because I could not travel without the vaccine card, I [was] allowed to take another Johnson Johnson jab in order to be allowed to continue with my journey. (Interview, May 2022)

In Mpondwe and Karambi PoEs in Kasese district, many villagers were forcibly vaccinated at makeshift roadblocks. Research participants recalled: 'On that day, every bus was stopped and soldiers demanded to see all passengers' COVID-19 vaccine cards. Whoever did not have one was forcibly vaccinated before being allowed to proceed' (Interview, July 2022). Another interviewee added, 'the people at the roadblock were very tough and cruel. It was a waste of time to argue with them. Although I had only left my COVID card at home, I simply allowed [myself] to be re-vaccinated' (Interview, July 2022).

In the Kasese towns of Mpondwe, Bwera and Mobuku, many small-scale traders, travelers and frontline health workers reported they were vaccinated three or four times within a month. The military, supported by Uganda Red Cross, Baylor Uganda, WHO and CDC Africa, set up roadblocks at strategic points and prohibited people from traveling unless they showed proof of vaccination. Any individual who had lost, forgotten or not received a vaccination card in the prior COVID-19 vaccination exercise was vaccinated again before being allowed to proceed. In Kasese and Mpondwe POE, travelers who resisted or questioned why they needed to be vaccinated for COVID-19 were arrested and delayed in conducting their business:

By the end of the campaign mode, many people had been vaccinated multiple times with Johnson & Johnson vaccine. They were forced to take the vaccine again if they had forgotten their cards; they were punished and then vaccinated if they resisted and many only accepted to be vaccinated because in the previous receipt of COVID-19 vaccine, they were not given vaccine cards as proof. (Interview, May 2022)

Vaccine hesitancy among frontline health workers

Many frontline health workers questioned the vaccine rollout and were hesitant to be vaccinated themselves; to protect their jobs, some secured vaccination cards without actually getting vaccinated. In Karambi, Bwera and Katwe health centers in Kasese district, vaccines were administered to villagers by VHTs, who would typically sit in an isolated tent, waiting for clients to present for vaccination. Both potential clients and health care workers were keenly aware that the vaccines were at the point of expiry, leading to demotivation. VHTs would monitor the expiry dates closely and inform the District Taskforce when vaccine stocks were approaching expiry dates. In two interviews at Karambi and Katwe health centers, VHTs recalled how the Kasese DTF had instructed them to stop administering vaccines within one week of expiry. Additionally, the DTF in Kasese conducted radio talk shows indirectly encouraging people to access more reliable vaccines, entreating them to access *either* cholera or COVID-19 vaccines.

The cholera vaccine seemed much better since it had a longer expiry date of four years. But for COVID-19 vaccine, some had only one or two weeks to expiry date. Generally, clients who choose cholera vaccine should not take COVID-19 vaccines. (Interviews, May and September 2022)

Local authorities and local clinicians' perspectives about COVID-19 likely also affected uptake by people at risk. In several health centers in Kasese, Gulu and Wakiso, clients rejected the vaccine if the clinician they trusted or regularly interacted with in their local language had not been vaccinated. In a meeting to assess vaccine uptake after the campaign mode vaccination drive in Kampala, which I attended in June 2022, WHO representatives noted the inability of frontline health workers, ministry staff and the NEOC to vaccinate even the small target of 5 million Ugandans: 'What really needs to be done', one WHO official asked aloud, 'to attain this small figure?'

As noted, vaccine enforcement ignited outrage and suspicion. Nurses, midwives and other clinicians in Kasese, Gulu and Wakiso feared the vaccines were expired, others did not see their relevance, and still more perceived the vaccine rollout as a form of authoritarianism. Initial attempts to enhance the uptake of vaccines, mediated by militarization from May to August 2021, were met by public outcry. Many people wrote to newspaper editors raising questions of the forceful means to improve vaccine uptake

(Abet 2021; Monitor 2022). In response, the state, supported by donors, enacted and formulated legal guidelines to support their efforts.

Campaign mode, February–September 2022

The legal package and guidelines, which enabled COVID-19 vaccines to be delivered in ‘campaign mode’ or heightened health promotion, was launched in February 2022. This activity was led by the MOH-UNEPI program, with support by the WHO, CDC-Africa and other vaccine donors. Its main aim was to heighten and enhance uptake of COVID-19 vaccines in both rural and urban areas. Campaign mode activities strategically targeted distant and rural districts like Gulu and Kasese, which experienced the lowest COVID-19 mortality rates and case positivity in Uganda. The MOH-UNEPI teams, accompanied by loud music systems, following an itinerary of pre-organized meetings in upcountry centers, aiming to convince listeners of the importance of the vaccine. There were funds for this activity and many officers at the MOH traveled to villages with the aim of improving vaccine uptake. One officer at UNEPI mentioned: ‘The activity was important but it was [a] race against time, meeting global partner demands which was more important. Some vaccines were about to expire, or they were even expired. But people in many districts did not respond well’.

As already described, in campaign mode, the routine supply chain for vaccines was ignored. Typically, the guidelines enabled MOH colleagues to supply vaccines outside of clinical settings. District response teams commissioned original songs to be composed, suggesting that COVID-19 was a ‘killer disease’, and repeatedly played this in public settings. Vaccines were distributed at funerals, playgrounds, weddings and cultural ceremonies, and in playgrounds, schools and churches. At one funeral in Unyama subcounty, Gulu, a COVID-19 vaccine team erected a tent next to the burial ground, and repeatedly made announcements that mourners should go and get vaccinated. By observation, no one responded; people were grieving, laying wreaths and eulogizing the deceased. Apart from the villagers and frontline health workers who were forcibly vaccinated, few people responded to announcements by vaccine teams requesting that they leave funerals, weddings or other festivities to get vaccinated. One VHT member in Bardege hospital observed:

We know that the COVID-19 vaccines are expensive, and the government spent a lot of money to buy them. Some donors

have also brought for us many vaccines. But the people here do not listen. They think vaccines are only for diseases which can kill people. They say they have not seen anyone die of corona. (Interview, May 2022)

In Gulu, the campaign mode team invited one cultural leader to serve as a model; he agreed to be vaccinated in public. After campaign mode activities concluded in the districts, with low adult turnouts, the remaining vaccines were deposited in health centers and frontline health workers were instructed to administer them on request. By observation, in turn, in Karambi and GRRH, the frontline health workers shunted the responsibility for these activities once more to VHTs – local untrained volunteer healthworkers who usually perform only sensitization work and promote awareness in villages by distributing posters carrying health messages.

Kasese district is largely known for opposing initiatives of the current ruling party, and had historically been punished for it. Here, villagers argued that the vaccine rollout was another means by which the state could assert its authority, not necessarily to ensure population wellbeing. They questioned why the state, which frequently neglects their everyday health needs (by not providing malaria drugs, or facilities for admitting patients, for instance) was pushing vaccination against this ‘disease of the radio’. One leader at Kasese sub-county reflected:

I have never seen a disease which attracted as much government attention as COVID-19 in all my 10-year tenure, even when people in this area only know it through radio announcements ... the frequent jingles. The other day, the COVID-19 team wanted to interfere with a burial in the village, saying the death was due to COVID-19. Villagers became rowdy and wanted to fight the burial team. There was chaos until police came to resolve the situation and allow the villagers to conduct a normal burial.

Through observation and interviews with frontline health workers and clients alike at designated points, turnout was low from May and July 2022; on some days nobody came to be vaccinated. Over the course of a week-long observation in three health centers in May, only five teachers reported to Bwera hospital, and two villagers presented at Karambi for vaccination. At both these centers, when the VHT informed them that the vaccine was available but vaccination cards were not, people who presented then declined vaccination: it was apparent that their decision

to get vaccinated was not influenced by the need to protect themselves from infection, or any COVID variant, but by the need to protect their jobs.

The power of metrics and the logic of policy

The campaign mode for vaccine rollout was a legally backed approach to enhance COVID-19 vaccine uptake in Uganda. But while vaccination uptake dramatically reduced the mortality rate globally, COVID-19 had not been experienced as a killer disease in Uganda, even though it was known to cause deaths among people who were elderly or had comorbidities. Ugandans' experience of the disease was that severe infection and mortality were rare and the disease was never perceived as lethal. Certainly it was regarded as far more benign than other diseases like polio, measles and diphtheria, for which vaccination was traditionally rolled out in this same enhanced mode. However, the campaign mode was concerned with more than disease prevention. It was also concerned with a need to meet global health partner metrics; the state's assertion of power and authority; proof of response to an urgency by distributing this life-saving medical technology to vulnerable people; and dealing with vaccine hesitancy among poor people.

In the drive for vaccine equity, global health agencies pushed for the delivery of vaccines to Africa. Africa CDC also made this a priority. 'Partners' from these agencies operating in Uganda were under pressure to drive the vaccine equity agenda and improve vaccination rates to meet global targets; as one officer at UNEPI reflected: 'Each time we are invited for meetings by our partners, one of the items on the agenda is *how to improve vaccine uptake*. It is not even asked whether we need the vaccine such as COVID-19 vaccine or not' (June 2022).

I attended several meetings between November 2021 and June 2022 at MOH and NEOC and in District Task Force meetings in Gulu and Kasere and at the Public Health Emergency Operation center. In one meeting, a senior officer at the WHO angrily asked: 'How are we going to present this report to our global partners? What haven't we done right to only attain these low figures of less than five million people vaccinated?' He continued:

Why are even nurses not able to input data digitally to show the number of clients they have administered COVID-19 vaccines to? Don't forget that we have invested so much in the digital platform and trained these very nurses! I hear in many health centers, the

task has been left to unpaid, less technical VHTs. (Global hybrid and Zoom discussion at Uganda Low Vaccine Uptake, June 2022)

Over time, it became clear that the government's intentions were as much oriented towards fulfilling the expectations of recording high COVID-19 vaccine coverage metrics predetermined by global health partners, as they were towards protecting people against disease. The power of data and its importance to government and donor officials emerged as a key force driving action in Uganda. Further, deaths reported during the pandemic were more often caused by underlying diseases including malaria than COVID-19. Meanwhile, immunization clinic attendance declined. The number of children treated for pneumonia, diarrhoea and malaria also declined (Burt et al. 2021). Immunization against diseases like diphtheria, tetanus and pertussis (combined in the DTP3 vaccine) was lower in 2020 than in the two previous years (Burt et al. 2021). Neonatal mortality rose from 39 to 49 per 1,000 live births. Burt and colleagues (2021) have shown that antenatal attendance decreased and remained low during lockdown in Uganda, with an average reduction of 370 per 1,000 pregnant women reporting for routine antenatal care (between March 2020 and December 2020). This was likely partly due to the lack of availability of public transport during lockdown, but also because, apart from essential workers, at-risk people – including pregnant women – were prohibited from traveling.

Some WHO officers strategized to improve metrics for COVID-19 vaccine uptake, though repeatedly questioning NEOC and MOH officials and various task force members' efforts during online meetings. NEOC and MOH officials typically suggested discussing another item on the agenda. For instance, in June 2022, one agenda item was a report on low uptake of COVID-19 vaccines to global partners. Instead of presenting a grim picture, WHO participants suggested a positive spin, with emphasis on 'what went well about COVID-19 vaccines in Uganda'. The report emphasized 'positives', including that 'the president and cultural leaders were supportive'. For instance, in many districts like Gulu, Kasese and Kyenjojo, some cultural leaders agreed to openly take the COVID-19 vaccine. But not even these 'performances' motivated villagers to come out and take the vaccines. The NEOC report, drafted during the above stakeholders' meeting with MOH officials, reached certain conclusions, including that campaign mode had not improved vaccine uptake. Further, all efforts to enhance vaccine uptake yielded limited success.

In several interviews, even with senior officers at the MOH headquarters in Kampala, respondents wondered why state efforts in COVID-19

containment were not commensurate with how COVID-19 manifested. One senior officer at the pharmaceutical department, for example, argued:

Viruses are always with us, and the idea that they can shift from latent to virulent mode should be common knowledge to us. The only thing is that we should know that they can be controlled through basic approaches such as isolation, quarantine and conservative management. The best way to do this is to first make sure our health systems are efficient and equipped. Therefore, vaccines are not the best approach. For instance, this COVID-19 vaccine. How many variants are you going to deal with? What other variants are still in latent phase? Are you going to have resources to keep manufacturing vaccines for every virus? (Interviews, MOH, June 2022)

As this suggests, many people felt that vaccination was inappropriate, given COVID's manifestation as a mild disease in the majority of the population. Further, people questioned the prioritization of resources. For one government official, investment in strengthening general health systems (whereby hospitals were equipped with staff, clinicians and equipment to handle infectious diseases) seemed a more valuable use of time and resources. From his point of view, Uganda's experience of COVID-19 failed to justify vaccinating its entire population.

Resistance to containment methods also manifested at a national level, as this MOH officer suggested:

This COVID-19 has exposed many things among local and policy makers here at the MOH. Can you imagine, because there was some money received from donors, some officers were being co-opted and requested to go for admission at Mulago to try to indicate a burden of disease in order to receive some donor money? All this in order for [the] state to show other global actors that Uganda was also affected by COVID-19. (Interview at MOH, May and June 2022).

In short, Ugandan officials tried to respond to donor interests with a view to keeping funds flowing. Proportionate disease containment responses or tailored national responses proved impossible because of the need to report to funders, demonstrating outputs required by international partners. One officer at MOH-UNEPI argued:

This COVID-19 vaccine thing is dividing clinicians, politicians and responders. People in UNEPI are always invited by say Pfizer

and Sanofi Aventis for conferences abroad. And they come back with such powerful ideas to support their plans that what is left is to accept their ideas. The way I see it, they already know how COVID-19 was experienced here. They know that those who died in hospitals are mostly those with other conditions and they were neglected. They also know that Uganda has not really experienced the virulent form of the disease like the Europeans, Americans and Chinese. But since they have been facilitated financially, they will have to support the idea of vaccination even when it is not necessary. Just follow closely, there is no way those officers can ever decline the administration of any vaccines. ... I told you my story, there are many things which happened during COVID-19 lockdowns that made every officer here [at MOH] not trust anything about COVID-19. The responders, the money for admission, the admission and subsequent neglect, the yearning to increase the number of COVID-19 deaths, *in order to justify budget increase*. (Interview at MOH, May and June 2022).

As this indicates, some Ugandan officers believed in the value of a more context-sensitive approach to a vaccination campaign in Uganda, one attuned to local emerging epidemiological data and existential reality. Many officials interviewed, including the officer at MOH-UNEPI, were aware that donor funds were being directed to the country because of an assumption that COVID-19 would play out in Uganda as it had in other countries. *Ugandan officials did not want this flow of funds to cease and so were keen to meet the expected outputs. Further, pharmaceutical companies were perceived to have vested interests in the vaccine push.*

In one regional meeting hosted by MOH-Entebbe in April 2022 with representatives from the CDC, WHO and COVID-19 support partners like the European Union, the issue of rolling out the COVID-19 uptake in campaign mode was discussed extensively. One officer recounted that ‘even if the CDC discovered that the COVID-19 vaccine was not accepted by local people, and that people needed something else ... still the money for the COVID-19 vaccine had to be used for the COVID-19 vaccine’. He continued ‘I would rather take back the money, instead of giving it to the country to allocate it to other pressing health matters’ (Interview, April 2022).

As this participant highlighted, there was an emerging disconnection between local and global stakeholders in health, but also between MOH officials and global health partners’ actions, informed by their assessment of Ugandan health priorities and their objective of achieving

greater global vaccine equity. The officials, aware of COVID-19's mild manifestation in Uganda, had a different understanding of priorities to which donor funds might be allocated: strengthening health systems or responding to other infectious disease outbreaks, for example. But the need to preserve funding flows made it difficult for health officials to push back against the global agencies, whose representatives sat in on the national meetings. One junior officer at the MOH concluded an interview by speaking of the skewed nature of the vaccination effort and focus of the campaign mode. He perceived vested financial interests behind this widespread vaccination drive, since, in his view, it was evident that COVID-19 vaccination should not be a priority for Uganda:

At this stage we are all aware that the West was more affected by this disease. This made the pharmaceutical companies invest so much money to ensure that the disease is eradicated. Do you really think the investor in vaccines can just sit back and deal with losses? At least there must be ways for them to breakeven with these commodities. For instance, we have seen even World Bank, giving us a debt of 18 billion to ensure that this vaccine thing is rolled out. (Interview June 2022)

Summary observations

Four key observations emerge from the research I have presented. First, perceptions and observations of COVID-19 led villagers and others to resent forced quarantine, and to resist accepting the COVID-19 vaccine, even when distributed in campaign mode. Second, there was widespread ambivalence about accepting and distributing COVID-19 vaccines among frontline health workers, and health policymakers expressed disquiet about prioritizing the roll out of vaccines. This highlights a perceived disconnection between global priorities and discourses, and experiences and priorities at national and district levels. A third and closely connected observation is that the pressures to succumb to the policies and priorities of global partners was perceived to be intense. The endeavor to meet global health partners' agendas and maintain funding streams for other global health programs in Uganda underpinned the shift from static to campaign mode. This shift demonstrates how a global political economy shapes the policies and practices of national governments, as they seek to maintain a good image to global donors to pharmaceutical companies. Finally, state authoritarianism in

efforts to enforce first quarantine then vaccination sharpened distrust among target populations.

These findings broaden our understanding of vaccine preparedness and response. By foregrounding the perspectives of health officials and practitioners at national, district and sub-county levels, I demonstrate the multiple ways in which politics in both global and domestic spaces shaped endeavors to contain and manage COVID-19.

I see contradictions and disconnections between Uganda's local experience of COVID-19 and the global response. Officials questioned the necessity of investing in COVID-19 vaccination for all citizens, including young people. As well as analysing these contradictions, I also share ethnographic findings from a national health policy level, from districts and hospitals and among villagers in our study sites. Although here I follow the experience of COVID-19, elsewhere with others I have shown similar disjunctures characterizing approaches towards Ebola containment in northern and southwestern Uganda (Park and Akello 2017, Akello and Parker 2022). For instance, in Park and Akello (2017), we have shown how at-risk groups practiced caregiving in spite of their awareness of Ebola's virulence, yet local global health campaigns emphasized social distancing, mask wearing and referring affected people to designated points of treatment. To a great extent, at risk people go beyond standardized approaches to provide care in Uganda. This is, in part, a result of their awareness of challenges in the health system (Akello and Beisel 2019). Despite Uganda's experience with lethal epidemics like Ebola, and despite prior learnings that tackling one disease at a time is not only costly but also diverts attention from other priority health matters (Akello and Parker 2022), during the pandemic, again, Uganda focused only on COVID-19, a focus that had little impact on improving general wellbeing.

In *Pathologies of Power*, Paul Farmer (2004) explained how global and economic powers are entrenched in approaches to deal with diseases of poverty, including recommending appropriate technologies. If over 80 per cent of cases of COVID-19 in Uganda were experienced as mild or asymptomatic, then the current campaign mode to enhance uptake of COVID-19 vaccines is a classic example of how pathologies of power manifest among impoverished populations to enforce attempts to assert power and authority. Such heavy-handed state activities resonate with what Fisher and Anderson (2015) term securitization of development, whereby Uganda may evoke security threats to enforce its authoritarian agenda. Some of these authoritarian activities may be financed by other global agendas.

Ugandans have different priorities, including vaccinating children against killer diseases, fighting hunger and reducing mortality due to diseases like malaria, HIV and maternal mortality. As one respondent reflected: ‘You have brought the COVID vaccine, but we want cassava.’

The contradictions and disconnects were unprecedented during the COVID-19 pandemic. They confirm that there are no conventional standard interventions in global health, and that past experiences and the political atmosphere shape people’s participation in and adherence to global health interventions. To ensure better preparedness for disease epidemics in Uganda, there is an urgent need for decentralized systems of prioritization and decision making, and by extension, I argue, for a decolonizing of global health.^{2,3}

Notes

- 1 PCR (polymerase chain reaction) test used to diagnose people carrying the SARS-CoV-2 virus, which causes COVID-19.
- 2 I thank my colleagues, Professors Lenore Manderson, Nancy Burke, Melissa Parker and Hayley MacGregor for their constructive comments. The study was funded by Wellcome Trust (NH/17033).
- 3 Ethics approval was granted by the Uganda National Council of Science and Technology (SS4895).

References

- ‘67,000 Covid-19 vaccine doses set to expire’. 2021. *The Independent*, 23 September. Accessed 27 October 2024. <https://www.independent.co.ug/67000-covid-19-vaccine-doses-set-to-expire/>.
- Abet, Tonny. 2021. ‘Covid-19 vaccines will expire in July, says Health Ministry’. *Monitor*, 21 April. Accessed 27 October 2024. <https://www.monitor.co.ug/uganda/news/national/covid-19-vaccines-will-expire-in-july-says-health-ministry-3370710>.
- Africa Centres for Disease Control and Prevention (CDC). 2020. ‘Majority of Africans would take a safe and effective Covid-19 vaccine’. Accessed 15 February 2021. <https://africacdc.org/news-item/majority-of-africans-would-take-safe-and-effective-covid-19-vaccine>.
- Agaba, Nicholas. 2021. ‘Uganda to receive second batch of Covid vaccines today’. *The Kampala Post*, 16 June. Accessed 27 October 2024. <https://kampalapost.com/content/uganda-receive-second-batch-covid-vaccines-today>.
- Ajuna, David Vosh. 2021. ‘Uganda: Museveni to medics – Return to work, risk allowances are in the bank’. *The Monitor*, 26 June. Accessed 12 November 2024. <https://allafrica.com/stories/202106260082.html>.
- Akello, Grace. 2020. ‘COVID-19 in northern Uganda: Resistance, defiance and hospitalization of asymptomatic cases’. *BMJ Global Health Blogs*, 9 July. Accessed 27 October 2024. <https://blogs.bmj.com/bmjgh/2020/07/09/covid-19-in-northern-uganda-resistance-defiance-and-hospitalising-asymptomatic-cases/>.
- Akello, Grace and Ulrike Beisel. 2019. ‘Challenges, distrust, and understanding: Employing communicative action in improving trust in a public medical sector in Uganda’. *SageOpen* 9(4). <https://doi.org/10.1177/2158244019893705>.
- Akello, Grace and Melissa Parker. 2022. ‘Confronting epidemics: The art of not knowing and strategic ignorance during ebola in Uganda’. In *Routledge Handbook of Public Policy in Africa*, edited by Gedion Onyango, 460–72. London: Routledge.

- Bongomin, Felix, Brian Fleischer, Ronald Olum, Barbra Natukunda, Sarah Kiguli et al. 2021a. 'High mortality during the second wave of the coronavirus disease 2019 (COVID-19) pandemic in Uganda: Experience from a National Referral COVID-19 Treatment Unit'. *Open Forum Infectious Diseases* 8 (11): ofab530. <https://doi.org/10.1093/ofid/ofab530>.
- Bongomin, Felix, Ronald Olum, Irene Andia-Biraro, Frederick Nelson Nakwagala, Khalid Hudow Hassan et al. 2021b. 'COVID-19 vaccine acceptance among high-risk populations in Uganda'. *Therapeutic Advances in Infectious Diseases* 8: 20499361211024376. <https://doi.org/10.1177/20499361211024376>.
- Bamulanzeki, Nicholas. 2021. 'Uganda to destroy expired Covid vaccines worth \$7m'. *Africa News*, 13 August. Accessed 14 November 2024. <https://www.africanews.com/2024/01/10/uganda-to-destroy-expired-covid-vaccines-worth-7m/>.
- Burt, Jessica Florence, Joseph Ouma, Lawrence Lubyayi, Alexander Omone, Lorna Aol et al. 2021. 'Indirect effects of COVID-19 on maternal, neonatal, child, sexual and reproductive health services in Kampala, Uganda'. *BMJ Global Health* 6 (8): 1–11. <https://doi.org/10.1136/bmjgh-2021-006102>.
- Farmer, Paul. 2004. *Pathologies of Power: Health, human rights and new war on the poor*. Berkeley: University of California Press.
- Fisher, Jonathan and David M. Anderson. 2015. 'Authoritarianism and the securitization of development in Africa'. *International Affairs* 91 (1): 131–51. <https://doi.org/10.1111/1468-2346.12190>.
- IntraHealth International. 2022. 'COVID-19 vaccine drives increase coverage in Uganda's eastern region', 16 August. Accessed 14 July 2024. <https://www.intrahealth.org/news/covid-19-vaccine-drives-increase-coverage-ugandas-eastern-region>.
- Jain, Jyoti, Suman Saurabh, Prashant Kumar, Mahendra Kumar Verma, Akhil Dhanesh Goel et al. 2021. 'COVID-19 vaccine hesitancy among medical students in India'. *Epidemiology and Infection* 149: e132. <https://doi.org/10.1017/S0950268821001205>.
- Kanyike, Andrew Marvin, Ronald Olum, Jonathan Kajjimu, Daniel Ojilong, Gabriel Madut Akech et al. 2021. 'Acceptance of the coronavirus disease-2019 vaccine among medical students in Uganda'. *Tropical Medicine and Health* 49 (1): 37. <https://doi.org/10.1186/s41182-021-00331-1>.
- Kyeyune, Hamza. 2021. 'Uganda's slow pace of Covid-19 vaccination sounds alarm bells'. *AA (Anadolu Agency)*, 18 October. Accessed 27 October 2024. <https://www.aa.com.tr/en/africa/uganda-s-slow-pace-of-covid-19-vaccination-sounds-alarm-bells/2395251>.
- Margini, Federica, Anooj Pattnaik, Tapley Jordanwood, Angellah Nakyanzi and Sarah Byakika. 2020. 'Case Study: The initial COVID-19 response in Uganda'. Washington, DC: ThinkWell and Ministry of Health, Uganda.
- Monitor (Editor). 2022. 'Expired Covid vaccines: We need a new strategy'. *Monitor*, 16 January. Accessed 27 October 2024. <https://www.monitor.co.ug/uganda/oped/editorial/expired-covid-vaccines-we-need-a-new-strategy-3684228>.
- Morrison, J. Stephen. 'Impact of COVID-19 in Sub-Saharan Africa'. Center for Strategic and International Studies (CSIS), 2020. Accessed 20 November 2024. <http://www.jstor.org/stable/resrep37623>.
- Museveni, Yoweri. 2020. 'Address on the corona virus (COVID 19.) Guidelines on avoiding the pandemic'. Speech transcript, 18 March. Accessed 23 May 2022. <https://www.yowerikmu.seveni.com/address-corona-virus-Covid-19-guidelines-avoiding-pandemic>.
- Ojambo, Fred, 2022. 'Uganda destroys expired virus shots on slow uptake'. *Bloomberg*, 15 January. Accessed 27 October 2024. <https://www.bloomberg.com/news/articles/2022-01-15/uganda-destroys-expired-virus-shots-on-slow-uptake-monitor>.
- Olum, Ronald, Jonathan Kajjimu, Andrew Marvin Kanyike, Gaudencia Chekwech, Godfrey Wekha et al. 2020. 'Perspective of medical students on the Covid-19 pandemic: Survey of nine medical schools in Uganda'. *JMIR Public Health and Surveillance* 6 (2): e19847. <https://doi.org/10.2196/19847>.
- Park, Sung-Joon and Grace Akello. 2017. 'The oughtness of care: Fear, stress, and caregiving during the 2000–2001 Ebola outbreak in Gulu, Uganda'. *Social Science & Medicine* 194: 60–6.
- Parker, Melissa, Hayley MacGregor and Grace Akello. 2020. 'COVID-19, public authority and enforcement'. *Medical Anthropology* 39 (8): 666–70. <https://doi.org/10.1080/01459740.2020.1822833>.

- Parker, Melissa, Moses Baluku, Bono E. Ozunga, Bob Okello, Peter Kermundu et al. 2022. 'Epidemics and the military: Responding to COVID-19 in Uganda'. *Social Science & Medicine* 314: 115482. <https://doi.org/10.1016/j.socscimed.2022.115482>.
- Sallam, Malik, Deema Dababseh, Huda Eid, Kholoud Al-Mahzoum, Ayat Al-Haidar et al. 2021. 'High rates of COVID-19 vaccine hesitancy and its association with conspiracy beliefs: A study in Jordan and Kuwait among other Arab countries'. *Vaccines (Basel)* 9 (1): 42. <https://doi.org/10.3390/vaccines9010042>.
- Sirleaf, Mitigai. 2021. 'Disposable lives: COVID-19, vaccines, and the uprising'. *Columbia Law Review* 121 (5): 71–94. Accessed 27 October 2024. https://digitalcommons.law.umaryland.edu/fac_pubs/1648.
- 'UMA raises concern'. 2020. 'UMA raises concern about delays to pay Covid-19 risk allowances to medics'. *Independent*, 11 December. Accessed 12 November 2024. <https://www.independent.co.ug/uma-raises-concern-about-delays-to-pay-covid-19-risk-allowances-to-medics/>.
- WHO (World Health Organization). 2021. 'Uganda receives 864,000 doses of COVID-19 vaccines'. *WHO Uganda*, 6 March. Accessed 27 October 2024. <https://www.afro.who.int/news/uganda-receives-864000-doses-covid-19-vaccines#:~:text=KAMPALA%2D%20Friday%2C%206%20March%202021,access%20to%20COVID%2D19%20vaccines>.

